

Cancer

Squamous cell carcinoma (SCC) may arise from mucosal lichen planus lesions (e.g., oral, vulvar, penile) and from hypertrophic cutaneous lichen planus lesions, particularly on the distal extremities.

A 1% incidence of SCC has been reported in patients with oral lichen planus. Proposed factors contributing to this increased risk include:

- Increased susceptibility of affected oral mucosa to *Candida albicans* and to exogenous mutagens from tobacco and alcohol.
- Chronic inflammation and repeated epithelial injury in oral lichen planus, which may promote genetic mutations leading to carcinogenesis.

Case reports describe SCC developing in hypertrophic cutaneous, anogenital, or esophageal lichen planus lesions. Persistent or non-healing ulcers should be biopsied, especially if they are resistant to standard therapy.

Systemic complications

Systemic complications may result from long-term use of medications used in lichen planus treatment, including:

- Infections
- Osteoporosis
- Adrenal insufficiency
- Bone marrow suppression
- Renal damage
- Hyperlipidemia

Therapeutic management

Objectives

Lichen planus is a chronic condition. The primary goals of treatment are symptom control, prevention of tissue damage, and improvement in quality of life. Therapy should be tailored to disease severity, with emphasis on minimizing adverse effects.

These guidelines provide treatment recommendations for various forms of lichen planus, aiming for the highest level of expert consensus. Except for topical corticosteroids, all other treatments are considered off-label.

Management of cutaneous lichen planus

The main objectives are relief of pruritus and acceleration of lesion resolution.

Topical glucocorticoids are the first-line treatment, despite limited evidence from randomized controlled trials. If topical therapy fails, **oral corticosteroids** are used. Oral corticosteroids may also be initiated early in cases with rapidly progressive or atrophic lesions.

First-line treatments

The following interventions are recommended for cutaneous lichen planus:

- **Topical steroids:** Superpotent or potent agents such as clobetasol propionate, betamethasone dipropionate, triamcinolone acetonide, or fluocinolone acetonide
- **Intralesional triamcinolone injections:** Especially for hypertrophic or treatment-resistant lesions (5–20 mg/mL every 2–4 weeks)
- **Systemic corticosteroids:**
 - Oral prednisone: 30–80 mg/day for 4–6 weeks
 - Intramuscular triamcinolone: 40–80 mg every 6–8 weeks, if lesions are unresponsive to topical therapy

- **Acitretin:** 20–35 mg/day
- **Isotretinoin**
- **Oral cyclosporine:** 3–5 mg/kg/day

Second-line treatments

Given that cutaneous lichen planus is often self-limited and complications are rare, treatment decisions must balance potential benefits against risks of adverse effects.

Second-line and third-line therapies are outlined in Tables 4 and 5 (not included here).

Management of mucosal-oral lichen planus

Oral mucosal lichen planus can be challenging to manage, especially when ulcerative or erosive forms are present. Historically, treatment has focused on palliation rather than cure, aiming to alleviate discomfort and prevent progression.